# Hip Examination — OSCE Checklist

Deakin MD2 · MM7 · Time: 8–10 min

*Tick each step as you perform or verbalise it.* **Shaded rows (*)** *are optional or done if indicated.* **†** *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests.** *If fracture suspected: passive movement with great caution, stop if painful or crepitus. Acute loss of circulation or sensation = emergency — don't delay management.*

## 1. Introduction

| | |
|---|---|
| ☐ | Wash hands; introduce self; confirm patient ID; explain and gain consent |
| ☐ | Position: start standing |
| ☐ | Exposure: underwear ideally (shorts for comfort); shirt lifted to waist; compare with the other side throughout |
| ☐ | Ask about pain before touching — "Where is it sore?" |
| ☐ | General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal? |

## 2. Examination

| | Step | What to do / say | Abnormal |
|---|---|---|---|
| **Look — standing** | | | |
| ☐ | Walking in | Walking stick or other aid | |
| ☐ | Gait | Normal? Antalgic (short stance on painful side)? Trendelenburg (waddling) † | Hip pain, abductor weakness |
| ☐ | Inspect front, back, sides | Deformity, scars/wounds, gluteal and quadriceps wasting, symmetry | Previous surgery, disuse |
| ☐ | Trendelenburg test | Stand on one leg (hold your hands †); watch iliac crests; repeat on the other leg | Pelvis drops on the lifted side = weak abductors on the **standing** side |
| **Look — supine** | | | |
| ☐ | Re-inspect | Anything missed; leg rotation or shortening † | Fractured NOF: shortened, externally rotated |
| **Feel** | | | |
| ☐ | Hip joint | Just distal to midpoint of inguinal ligament; watch the face | Joint tenderness |
| ☐ | Greater trochanters | Palpate both | Trochanteric bursitis |
| **Move — supine (active, then passive if pain or restriction)** | | | |
| ☐ | Flexion | Straight leg raise (to 90°); knee bent (to 120°) | |
| ☐ | Thomas test | Hand under lumbar spine; flex one hip fully until back is flat; watch the other leg | Other leg lifts = fixed flexion deformity |
| ☐ | Internal / external rotation | Hip and knee at 90°, rotate foot out (internal) and in (external) † | Internal rotation lost first in OA † |
| ☐ | Abduction / adduction | Stabilise the pelvis with your other hand on the opposite ASIS † | |
| ☐ | Extension | Patient prone: lift leg off bed | |
| **Measure** | | | |
| ☐ | True leg length | ASIS → medial malleolus, both sides | Difference = bone shortening (fracture, congenital, surgery) |
| ☐ | Apparent leg length | Umbilicus → medial malleolus, both sides | Difference = pelvic tilt, adduction/abduction, spinal curvature |
| **Special tests (confirm done)** | | | |
| ☐ | Trendelenburg, Thomas, leg lengths | Usually completed in Look / Move / Measure | |

## 3. Closing

| | |
|---|---|
| ☐ | Thank patient; help them dress; wash hands |
| ☐ | Completion: "I would examine the lumbar spine and knee (joints above and below) and do a neurovascular exam of the lower limb †." |
| ☐ | Present: "Normal gait with no aids. No deformity, wasting or scars; Trendelenburg negative. No tenderness. Full range of movement; Thomas test negative; true and apparent leg lengths equal." |